

SKIN ULCER OR NON-HEALING WOUND: DIAGNOSIS

Give urgent attention to the patient with a skin ulcer or non-healing wound and any of:

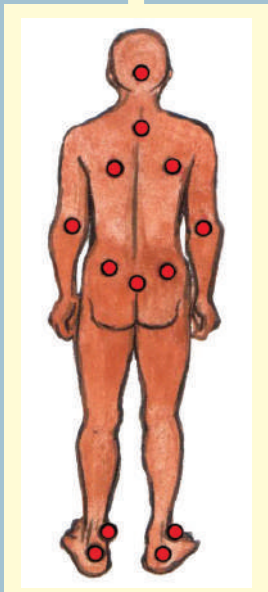
- Infection (surrounding skin red/warm/swollen) with any of: BP < 90/60, pulse > 100, temperature $\geq 38^{\circ}\text{C}$, confused, blisters, crepitus¹ or severe pain
- Sudden severe leg pain at rest with any of the following in the leg: numbness, weakness, pallor, no pulse: **acute limb ischaemia** likely

Refer urgently.

Identify cause of skin ulcer or non-healing wound

- If genital ulcer → 49.
- Is patient bedridden/in wheelchair and is ulcer in common pressure ulcer site (see below)?

Yes



Pressure ulcer likely

No

Is ulcer on the lower leg or foot?

Yes

Does patient have muscle pain in legs/buttocks on exercise or reduced/absent foot pulses?

Yes



Source: Francois Coetzee

Arterial (ischemic) ulcer likely

No

Is there darkening of skin around ulcer, varicose veins or chronic swelling of the leg?

Yes



Source: University of Cape Town

Venous ulcer likely

No

Is patient known with diabetes?

Yes



Source: BMJ Best Practice

Diabetic ulcer likely

No

- Ask about previous trauma, surgery, skin rash, burn, cancer or radiation therapy.
- Exclude diabetes: check glucose ↗ 17.
- Test for HIV ↗ 110.
- If cough ≥ 2 weeks, weight loss, night sweats or fever ≥ 2 weeks, exclude TB ↗ 92.

- Give routine care for ulcer/wound → 76.
- If pressure ulcer, also arrange for doctor review.
- If arterial, venous or diabetic ulcer, or cause uncertain, also refer patient to district hospital for further assessment.

¹Crepitus is a crackling, popping or grating sound or feeling under the skin.